ED NURSE SEPSIS EARLY MANAGEMENT WORKFLOW ALGORITHM

SBC - KP Fontana Medical Center / KP Ontario Medical Center

Age ≥18 y.o. with Suspected infection

2+ SIRS CRITERIA:
- T<96.8 F or >100.9 F
- HR>90
- RR>20
- WBC <4K or >12K or >10% bands

Sepsis Triage Screening / BPA Alert: Select ***OPEN ORDER PANEL***

SEPSIS ORDER PANEL: Select **LACTIC ACID, CBC/ DIFF, BLOOD CULTURES x 2**, and notify attending physician

Initiate ***SEPSIS ALERT*** per ED protocol

CMS 1+ ORGAN DYSFUNCTION CRITERIA:
- Two SBP<90 / MAP<65 or SBP ↓>40 mmHg w/in 3 hrs. of each other
- Acute Respiratory Failure – New need for invasive / non-invasive ventilation (BiPAP or CPAP)
- Cr >2 or U.O. <0.5 ml/kg/hr x 2 hrs.
- Total Bilirubin >2 mg/dL
- Platelet <100K
- INR >1.5 or aPTT >60 sec
- Lactate >2 mmol/L

qSOFA CRITERIA:
- Altered mental status (GCS<15)

SSP1 CRITERIA:
SEVERE SEPSIS PRESENT
- Suspected infection
- 2+ SIRS Criteria
- 1+ Organ Dysfunction Criteria w/in 6 hrs. of each other

SEPSIS SMART PHRASE:
.SEPSISNURSINGNOTE

☐ ≤ 1 HR.
Draw ***INITIAL LACTATE*** ≤1 hr. of SSP1

☐ ≤ 3 HRS.
Collect ***BLOOD CULTURES*** prior to first antibiotic administration ≤3 hrs. of SSP1

☐ ≤ 3 HRS.
Administer monotherapy ***BROAD SPECTRUM ANTIBIOTIC**** ≤3 hrs. of SSP1. If not ordered, consult physician

***SBC BROAD SPECTRUM ABX MONOTHERAPY**

TRADE NAME	GENERIC NAME
Rocephin	Ceftriaxone
Unasyn	Ampicillin / Sulbactam
Zosyn	Piperacillin / Tazobactam
Fortaz	Ceftazidime
Maxipime	Cefepime
Claforan	Cefotaxime (not in pyxis)

Add'l Abx for gram + / anaerobic coverage may be added if clinically indicated to these single agents

Initial lactate level 2.1-3.9 mmol/L

SSP2: Initial lactate level ≥4 mmol/L

SSP2 CRITERIA:
SEPTIC SHOCK PRESENT
- Severe Sepsis & Persistent Hypotension

OR
- Severe Sepsis & Initial Lactate ≥ 4 mmol/L

INITIAL HYPOTENSION
Two SBP <90 / MAP <65 or SBP ↓ >40 mmHg w/in 3 hrs. of each other?

Yes →

☐ ≤ 3 HRS.
Administer ***CRYSTALLOID IV FLUID BOLUS (30 ML/KG)*** ≤3 hrs. of initial hypotension / SSP2 and document ***IV FLUID COMPLETION TIME*** on the ***MAR***

☐ ≤ 1 HR.
Obtain ***TWO COMPLETE SETS*** of ***VITAL SIGNS*** (T, P, RR, & BP) 15 mins. apart within ***1 HR. of IV FLUID COMPLETION TIME***

SSP2:
PERSISTENT HYPOTENSION
(Two consecutive SBP <90 / MAP <65 or SBP↓ >40 mmHg) ≤1 hr. of IV fluid completion time?

Yes →

☐ ≤ 6 HRS.
Administer ***IV VASOPRESSOR*** ≤6 hrs. of SSP2

Reassess ***VITAL SIGNS*** w/in 1 hr. of initiating IV vasopressor

No →

Standard Sepsis Therapy

No

☐ ≤ 3-6 HRS.
REPEAT LACTATE ≤3 hrs. of first sample collection time (≤6 hrs. of SSP1)

☐ ≤ 3-6 HRS.
REPEAT LACTATE ≤3 hrs. of first sample collection time (≤6 hrs. of SSP1)

ESCALATION PROCESS:
For any sepsis metric not ordered, please notify the physician, charge nurse, and ED ACD for corrections
For any sepsis metric still not ordered, please notify Sepsis Physician Champions:
ED – Michelle Sperry, MD 909-329-9863 / IM – Wei Chan (Milton) Hwang, MD 909-781-9252
Sepsis Coordinators: FMC - Jennifer Platt, RN 909-302-6891 / OMC - Judy Curran, RN 909-724-3559

LEGEND:
SSP1: Severe Sepsis Presentation
SSP2: Septic Shock Presentation

7/1/2020